

COMPANY DEEP-DIVE

Former Director at QuidelOrtho Pessimistic About Post-Merger Growth Amid Competitive Pressures

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SMART SUMMARY

Summary

The interview covers the company's product portfolio, market positioning, and the commercial and internal impacts of a major merger in the diagnostics industry. Discussion includes challenges in molecular and cardiometabolic testing, competitive dynamics, and organizational changes post-merger.

EXPERT BACKGROUND

- Former Director with direct experience overseeing commercial operations and product launches across both legacy organizations following a major diagnostic industry merger, with specific involvement in EMEA markets and point-of-care testing platforms.

EXPERT SENTIMENT

- The expert is pessimistic about the company's future growth prospects, citing declining sales, failed product launches, and increased competition.
- The expert believes the company lost its competitive edge in molecular point-of-care testing due to delays and technical failures, and sees little opportunity for regaining momentum in this segment.

TAKEAWAYS

- The company's molecular point-of-care testing platform, Savanna, was phased out after significant technical and commercial failures, leaving the company with an outdated and uncompetitive molecular offering.
- The cardiometabolic Triage platform, while robust and reliable, is considered obsolete, with component shortages and increasing competition threatening its future viability.
- The Sofia immunofluorescent platform is competitive but lacks unique selling points, facing strong competition and price pressure in the market.
- The company maintains a strong position in immunohematology and blood screening, but this market is mature, slow-growing, and unlikely to provide significant future growth.
- The merger resulted in little immediate change for customers but led to significant internal restructuring, with most layoffs and cultural shifts impacting the smaller legacy organization.

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Expert Bio

Former Commercial Director at QuidelOrtho, leaving in June 2023. Can speak about industry trends particularly involving Molecular Biology and Microbiology tests. Former Commercial Director at QuidelOrtho, leaving in June 2023. Reporting to the VP, the expert was responsible for overseeing all commercial operations within QuidelOrtho's Italian region. The expert is responsible for leading the first and largest launch of Savanna within QuidelOrtho's history and helped sustain the highest COVID-related revenues from among all of QuidelOrtho's territories. The expert led a team of vendors, sales agents, and marketing managers, and worked on microbiology and molecular biology tests within the firm.

Interview Transcript

Client ▶ 00:00:00

Hi, thank you for taking this call. Let's get started with market positioning and product focus. Where does **QuidelOrtho's portfolio resonate most strongly** with customers today? Where do you think it struggles versus competitors?

Expert ▶ 00:00:17

Okay. The merge of the two companies, Quidel and Ortho, former Ortho Diagnostics brought to a company that has a portfolio basically built on four main pillars or four main areas. **The Ortho, the former Ortho legacy product that are clinical chemistry instruments and immunochemistry, the core lab of the business and the transfusion medicine and blood donor screening from the Ortho side and from the Quidel side.**

Quidel was a company born as a rapid testing company. In particular, dedicated to microbiology tests. With the acquisition of the Triage business from Abbott, Quidel also integrated a cardiometabolic platform, POCT platform for cardiac tests. Quidel legacy products were focused on microbiology and cardiometabolic testing in a format typical of rapid testing with immunoassay technology, rapid test with the immunoassays and more recently, rapid testing for POCT with molecular technology.

We start speaking about the Quidel legacy products. Basically, the Quidel business is based on three platforms plus a few manual rapid tests. The immunometric test platform that is namely Sofia instrument, the cardiometabolic rapid testing for cardiology, that is the Triage platform purchased by Abbott, that purchased before from Alere and before Alere from Biosite. Triage is a very old platform. It's more than 25 years that is on the market.

The last one that was introduced as a molecular platform for rapid testing was named Savanna. Savanna is a rapid testing platform for multi-testing cartridges. **The problem** here is that the Savanna platform was intended to be the future of the company in the molecular area. I was in the company three, four years ago. The platform with one cartridge, a syndromic testing for four parameters, Flu A, Flu B, RSV, and SARS, COVID testing. This first cartridge was intended to be the first of a long series of different cartridges for other tests like sexual transmitted diseases, gastric panel, low and high respiratory tract panels, and many others. That was the original plan.

Unfortunately, the cost and the efforts and the time to develop new cartridges, at the end, **the result of this was failure.** It was a failure because it took too long time. The cartridges production was too manual and too expensive. The timing to develop new cartridges was too long and too demanding. **The instrument was not stable and reliable as it was supposed to be.**

On paper, **the product was very good, in terms of specification on paper.** In practice, for example, in Italy, I was the country that sold more than all the rest of the world together with this platform because **we gained a couple of big customers for this platform.** We were the first country to install many units in different hospitals, but after the installation, **we struggle a lot with technical failures,** reliability of the instruments, changes in the hardware that were needed to make it working. The first cartridge that was introduced remain the first and the only one.

At the end of the story, probably the new board that came in, when the old board, the one that took care of the fusion of the two company was practically all fired, dismissed, removed from the board. **The new board and the new CEO, I never met him, I don't know exactly, but at the end, they decided to abandon this project** completely.

Today, the molecular project for them is a phase out project, Savanna is a phase out and they bought a new company, if I'm not wrong, is based somewhere in Australia that has a project and a new platform in the pipeline to introduce molecular point-of-care testing with different cartridges. Practically, they decided to abandon the original molecular project and they started from scratch.

Today, the situation is that Savanna is a phased out product. This new alternative from this new company that they purchased for something like \$100 million, if I'm not wrong, is still to come. They have now almost, let's say, nothing from the point-of-care testing molecular business.

Before Savanna, they used to sell and they still selling molecular test, just kits to be applied to other companies platforms that is a product line that is called Lyra. They also have another system that is called Solana that is not a real-time PCR, is a endpoint PCR. **It's another technology much older and less performing than real-time PCR. At the end of the story, the offer that they have for molecular today is old and not competitive at all. Waiting for this new instrument from this new company that they acquired.**

The problem is that the time is passed and today is not like five years ago. **The market is not anymore in the pre-COVID condition where only few companies had available point-of-care in molecular testing systems.** Today, there are a lot of competitions, very aggressive and with a large portfolio of tests available and good technology or technological performances and features.

Therefore, **the market competition situation** is much more, let's say, clouded and full of alternatives than five years ago. **I have to say that with this fail of the Savanna project, they lost the momentum and the right time to be effective in this business.** Their presence in this the molecular business point-of-care is almost negligible today.

In the cardiometabolic testing, **Triage is a good platform. It's a very robust and reliable platform. The problem is that it's very old. Even the components and the spare parts are not available anymore.** To keep this instrument going and to keep running in the market, **they have now to try to save and ship back all the old instruments available to use and to cannibalize the old instrument** to take the components that are not produced anymore from the electronical companies.

Is **a good platform**, very well performing, very robust, but is so old that the Triage future is something that will not last for many years from now on because they have to find a new platform and replace the Triage platform as soon as possible.

They have a good high sensitive troponin test in this platform. As a matter of fact, was the very first high sensitive troponin POCT available in the market. Siemens came with alternative solution for high sensitive troponin. Now, there are also some other newcomers that are approaching this situation in this market. Also, **this market will become more and more competitive in the next coming years** and is another reason why they have to find an alternative platform for Triage.

There is the third platform that is named Sofia. Sofia is an immunofluorescent reader with several cartridges for different tests such as Legionella, Streptococcus pneumoniae, Campylobacter, Clostridium, and some other microbiology infectiology test. Is a good reader, nothing special, there are many others. The quality of the test and the cartridges is not bad, competitive with others.

Also in this area, there are several now alternatives with the different platforms and different instruments. Some of these instruments are more automated. For example, the SD Biosensor that is a Korean company, they have two options of instruments. One is simple for one single cartridge at one time, but they have also an alternative instrument where the user can upload in continuous loading more cartridges, one after the other. This brings to a walk away automation that is very much appreciated by the users.

SD Biosensor is very good and very competitive in the market, but there are also other companies like Abbott or Meridian or Siemens itself that they have alternative instruments in this microbiology area. In this area, they have good products, but with not so many unique selling points. They are very much me too products. **They struggle sometime with dumping pricing** to gain some market.

In fact, the sales in general, of course the sales for COVID testing, but in general, sales of Quidel legacy product are not growing after **the COVID time are decreasing year after year**. If you look the turnover results that they publish in the communication to the stock market, you can see that this is an area of continuous slight decrease. They do not have any particular expectation or announcement for new technologies in this area, not really. This is the Quidel part of the business.

In the Ortho part of the business, as I said, they have clinical chemistry, immunochemistry analysis, the Vitros analysis, and they have the Vision analyzers for immunohematology and blood screening. In this last market, immunohematology and blood screening, **they are historically one of the main leaders in the world**. They share market share basically with two other companies there, Grifols and the Bio-Rad. They have a leadership proposition. In several countries, they are market leaders. In some others, they are not market leaders, but **they are very good in terms of shares**.

This is a market very much, let's say, consolidated and established. There are no new technologies that can break through this market or change the general picture. In fact, this is a market that is not growing or slightly growing with let's say 2%, 3%, 4% per year. They keep going in this market, **they keep their market share**. They are not growing more than the market, they are not gaining significant market share from competitors, but they have a good product line, very well accepted historically by the customers. **They defend very well their own position in this area**.

In the clinical chemistry area, in clinical chemistry and in immunochemistry, the Vitros proposal has some specific characteristics compared to the others. The basic and the main point here is that in the core lab market, they have to compete with the big four, Roche, Siemens, Abbott, and Beckman Coulter that in this clinical chemistry, **immunochemistry segment are really very strong, very competitive with high level of technologies**, high level of instruments and high throughput, high speed, full automation, very good offer.

With the Vitros instruments, the Vitros instruments that are dry chemistry and not wet chemistry, **the Vitros instrument are not competitive with this solution in terms of throughput**. **They cannot compete for the medium high segment of this market because are too slow and too small for the number of tests** that need to be processed every day. Therefore, **it is very rare and very difficult** that QuidelOrtho can take over customers from these four companies is really difficult.

In some market, in some countries, **their presence in this segment is almost zero, really negligible**. In some others, if they try to compete in the small, medium segment, they can success somehow because the instruments are very well working, very precise, very accurate.

The main unique selling point that they use in the communication and in the approach to the customers is the fact that their technology is a dry chemistry technology and they do not use water for washing and pouring needles, washing cycles of the instruments. They do not have the normal waste of water that the other instruments have. They claim to be environmental friendly, they don't waste water. As a matter of fact, they don't use water, but they do a lot of solid waste because the cartridges for the test are solid and they have to face with the cartridges. They don't have water waste, but they have a lot of solid waste.

Sometimes, this is well accepted by the customers. Also, precision is a very good precision. They do not have specific advantages in terms of portfolio because the test that they have, **they are also competitors** have, and they are not particularly cheap or low-cost because the dry chemistry cartridges has a significant cost. From the pricing point of view, they are not particularly competitive.

They have also to face not only with the big four that I told you before, but with a lot of newcomers, especially from China and Korea that

are becoming more and more aggressive in the medium low segment of clinical chemistry, immunochemistry such as Mindray and Snibe and some other Chinese manufacturer. In fact also in this area, **the business and turnover is not growing**. It's stable, it's more or less stable.

All in all, as a company, considering all the products they have, I do not expect that **they can grow significantly**, not even in the future. From the respiratory side and the blood transfusion side, **they are losing market, they are losing turnover** because the market is decreasing. All in all, if you look on their results also in the last years, the stock value and the turnover are going down. This is basically the trend that I can expect also for the future for this company. That is my thinking.

Client ▶ 00:14:30

Very clear. I appreciate the detail and the perspective there. I do think it's consistent with what we've heard elsewhere about not necessarily to regain COVID revenues. With that, which customer type do you think still value Quidel legacy products most? For example, is it the hospitals, is it the private labs, is it the distributors? Do you think it's the right strategy?

Expert ▶ 00:14:55

The Quidel legacy, the customer target they have change a little bit country by country. For example, if you speak about Europe, maybe is not the same if you speak about U.S. because the U.S. segment and market are different from Europe. IN Europe, you have a doctor office or point-of-care like doctor office countries, but you have other countries where you don't have doctor office.

For example, the Sofia products or the Savanna products in Italy or Spain or France or even the U.K., the target were hospitals, microbiology hospitals and emergency departments. Emergency departments for the screening of the patient access to the hospital with COVID or flu. This is a point-of-care that can be used in the emergency department.

When we were installed in Savanna, our Savanna were installed in the emergency departments to screen patients before the access to the hospital. Sofia was installed in the microbiology labs because in Europe microbiology, very often is a specific lab, so is separated from the core lab. Immunochemistry, clinical chemistry, methodology lab is another lab. Microbiology is a different lab very often, especially in the medium big hospitals.

In this microbiology lab, they use rapid testing for routine. Even if it's a point-of-care, if the routine is a few samples per day, you don't need the big automation or large automation. You can do the routine. If you have a test like chlamydia, five tests in a day or 10 days, 10 tests in a day, you can use a simple point-of-care, one-shot cartridge, one-by-one to do the routine of the lab because it takes, let's say, 10 minutes per test, 15 minutes so you can manage with the point of care.

One typical target was the point-of-care. Sofia was the microbiology. For Triage, cardiometabolic was the emergency department and the cardiology department for high sensitive troponin and for BNP specifically. In some other countries like in Germany, for example, where you have doctor offices with small labs, you could target also these doctor offices with Sofia and also with Triage, you could target these doctor offices the same you can do in the Scandic countries and in Switzerland because these countries also have doctor offices segment.

Not in Italy, not in Spain, not in Greece, not in France. Very relatively small in U.K. Instead in U.S., you can use Sofia and there's Triage for doctor clinics, for private clinics, for private healthcare facilities. You can use it very easily It depends on the country. The rest of the world, I don't know. I don't have experience to tell you what is going on in Japan. In Japan, they have doctor offices. This is something I know. Japanese market, I know a little bit, but only Japanese market. Not in China. I have no idea what is going on in China.

Client ▶ 00:18:06

Thank you. With that, how effective is QuidelOrtho's distributor heavy model in EMEA compared with competitors that rely more on direct sale?

Expert ▶ 00:18:16

I have to say QuidelOrtho in the western European countries, in the main European countries, they are acting direct. They have direct organizations. They don't have distributors. They have distributors in, for example, Italy, Spain, Portugal, France, U.K., Benelux, Germany, Austria, Switzerland. These are Scandic countries. Russia, these are direct, only direct. If you consider other EMEA countries like East Europe, like Bulgaria, Romania, Czech Republic, or Poland, and the Middle East, like Iran, Iraq, Egypt, in those countries, they act with the distributors.

Some of them are very good performing, some others less. There are some direct employees in quite a lot to take the relationship and control the activities in those countries through distributors. I never had these responsibilities. I know the people that used to do that, but not me.

I know that **they are quite successful in some countries of those**, in some others less. It's not the point of if it's more successful a direct

organization or a distributor. The point is who is the distributor and how much is effective the distributor that you choose. Is the quality of the distributor that very often makes the difference at the end.

Client ▶ 00:19:45

Got it. That makes sense as well, just given the dynamics I've heard about within the space.

Expert ▶ 00:19:51

In the countries that I listed before, they are direct. They have direct people. In Italy, they have agents. Agents is a specific role that is only in Italy. Agents are not direct employees, but they work very closely with the direct organization. Is not a distributor, is something different. Are single private persons that are not employees, but work like an employee. This is in Italy, we call it agents. This is very specific for Italy. Nobody understand this figure outside Italy because this is so specific that is very difficult to understand.

Client ▶ 00:20:34

Okay. I appreciate the call out there. Looking briefly at the merger, from the commercial side, how did the QuidelOrtho merger actually land with customers and partners? For example, did the customers see more value or more confusion? Were there disruptions in coverage or messaging?

Expert ▶ 00:20:56

In the first two years after the merging, from the commercial point of view and the customer interface point of view, nothing changed because they kept the organization completely separate. Customers continue to see the same people doing the same things, changing the name from Ortho to QuidelOrtho or from Quidel to QuidelOrtho, but in practical terms, nothing changed. They continue to act with different people going to different targets.

The targets were, as I said before, not overlapping, were completely different. The target of the Ortho side was blood banking or transfusional labs and the central lab and the target of Quidel was microbiology, emergency department, cardiology departments, and doctor offices. There was no overlap at all. The sales rep that was Quidel continue to be Quidel, the sales rep that was Ortho continued to be sales rep of Ortho. Nothing happened in front of the customers.

At least up to now, after four years from the integration. Now, because **sales are going down anyway, they need to consolidate and integrate areas because sales are decreasing**. Integrating area and keeping two people in the same area going to different segment is something that is a large investment in terms of resources. Now, they are starting to give responsibilities to one sales of all the product portfolio, but until now, nothing was changed in front of customers.

Client ▶ 00:22:26

Got it. From an internal standpoint, what has that looked like? Has there been a cultural impact? Have there been many layoffs? What exactly does that look like?

Expert ▶ 00:22:40

Yeah. From the internal endpoint, **this was a very anomalous and very strange acquisition** because the company that acquired the other one was much smaller. The company acquired was the bigger one. When this happened, in many countries, the ratio between the two organization was one to 10, one Quidel, 10 Ortho despite Quidel acquired Ortho. Particularly in the internal office, back office functions and organization, the Quidel teams were very small. They didn't need big organizations.

Practically, the Ortho people and the Ortho organization took over almost everything. In fact, several people were laid off. What was this mission of many people was from the Quidel side. They took over the control of the back office and supporting activities on almost everything like logistics, quality assurance, regulatory, legal tenders, standards offices, those kind of things.

Client ▶ 00:23:43

Okay. What has this sentiment been? Has that led to any departures? Has the culture shifted? I'm just curious as to what the net impact has looked liked.

Expert ▶ 00:23:56

On the people?

Client ▶ 00:23:58

Yes, on the internal organization.

Expert ▶ 00:24:01

The Ortho people just continue to act as they were acting before. They were acquired and they were purchased, but at the end of the story, nothing changed for them. They just continue to act and to behave like before without any specific or any particular issue or change. For the people, it changed a lot because many were dismissed or fired or the position were closed. For example, my position. The ones that remain working also, they had to change completely their own way of acting, working there, behaving there, approaching customers. The business model was completely different. They had to adapt themselves under this, let's say, new style.

The biggest or the largest impact was for Quidel people. Even the board. When they changed the board because things were not going as expected two years after the acquisition, **who was dismissed** was the CEO. That was the Quidel CEO, was the chief financial officer, CFO. That was the Quidel CFO. That was the final happy story that was not so happy.

Client ▶ 00:25:13

Got it. This has been very helpful. Thank you. Have a great day.